

TGA Senate QON Response: Gap Analysis Against Commitments and Standards

Senate Questions on Notice: SQ13–19 (December 2025)

Context: NCIRS 'Surveillance of adverse events following immunisation in Australia, COVID-19 vaccines, 2022'

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Governance Hierarchy

The commitments examined in this analysis operate within a four-layer governance hierarchy. Each layer creates obligations for the layer below. Failure at the control activity level represents a breach of all higher layers.

Layer	Standard / Requirement	TGA Position	Failure Type
1. Governance Commitment National Cabinet Policy (Nov 2020)	"TGA will actively and comprehensively monitor any COVID-19 vaccines for safety" (p.13). "Pharmaco-vigilance Plan" anticipated (p.13, 15). PGPA Act s.17 requires accountable authority to govern entity.	15 VSIGs convened reviewing approximately 50 cases out of 1,053 reported deaths (4.7%). 95.3% of reported deaths have no documented expert assessment pathway. Senate Estimates (Oct 2025): monitoring conducted as "day-to-day processes" not tracked against Plan objectives.	Strategic misalignment. Control activity does not deliver governance commitment. Consistent with ANAO findings on risk-based targeting and incomplete performance indicators (CPS 107).
2. Control Framework TGA Safety Monitoring Plan (Feb 2021)	20 systematic outputs operationalise policy. Strategy 6 (p.5): identification and investigation of deaths. Strategy 7 (p.5): causality assessment of AEFI of special interest. Output 2.2 (p.8): investigation and clinical assessment. Output 3.1 (p.10): regulatory action.	15% of 20 outputs fully documented. No systematic tracking against objectives confirmed by TGA officials in the Senate. VSIG selectivity bypasses framework for 95.3% of deaths including 0/9 child deaths.	Operational deficiency. Framework exists on paper but execution not evidenced. Consistent with ISO 31000 requirement for integrated risk framework aligned to objectives.
3. Control Activity VSIG (FOI 4029-06)	Selective tool within Output 2.2: convened for "serious, unexpected, no obvious non-vaccine cause" (pp.3–4). Work Instruction states youth deaths "had potential to change benefit-risk balance."	15 VSIGs convened / 1,053 deaths (4.7%). 0/9 child deaths referred despite meeting Work Instruction criteria. No documented alternative assessment pathway for remaining 95.3%.	Systemic under-delivery. Control activity does not meet framework requirements. Internal criteria contradicted by practice. ISO 19011 requires evidence-based, risk-proportionate audit.
4. Reference Controls International standards	ICH E2E §3.1.3 (p.7): documented action plan for each safety issue with milestones. §3.1.4 (p.7): summary of actions completed. CIOMS VIII Ch.3 (pp.13–28): documented signal taxonomy. Ch.4 (pp.29–56): systematic data sources.	No milestones documented against Plan objectives. No signal management audit trails for 91 of 148 signals. No documented integration of active surveillance data sources.	Non-compliance with international standards TGA formally adopted as reference framework for the Plan. Non-binding but cited as benchmark.

Control failure: activity layer (3) under-delivers framework (2), breaching governance commitment (1). 95.3% of reported deaths have no documented assessment pathway against any layer.

The Questions

The NCIRS 2022 surveillance report documented 160 deaths from 18,398 AEFI reports for COVID-19 vaccines. Senator Antic identified:

- Six children (0–17): median vaccination-to-death interval 4.5 days
- Five young adults (18–29): median vaccination-to-death interval 3 days
- Only 1 of 160 deaths determined as likely linked to vaccination

Seven questions were asked (SQ13–19):

1. SQ13: Were the six children referred to VSIG? If not, why not?
2. SQ14: Were the five 18–29-year-olds referred to VSIG? If not, why not?
3. SQ15: Do causality assessment reports exist for each child death? Provide copies.
4. SQ16: Do causality assessment reports exist for each 18–29-year-old death? Provide copies.
5. SQ17: Why were only 2 of 160 fatal cases in 2022 referred to VSIG?
6. SQ18: How many of 1,053 DAEN deaths have had causality assessments?
7. SQ19: How many of 1,053 DAEN deaths been referred to VSIG, and where can reports be accessed?

Key Findings from TGA's Response

- 1,053 deaths reported on DAEN. TGA did not state how many received formal causality assessments.
- Approximately 50 cases reviewed by VSIG out of 1,053 deaths (4.7%).
- 15 VSIGs convened across the entire COVID-19 rollout.
- 9 child deaths on DAEN. Zero referred to VSIG.
- 1 of 5 young adult deaths (18–29) in 2022 referred to VSIG.
- Zero causality assessment reports released. Privacy cited.
- 9 prior SQONs (2023–24) cited as 'previous answers.' None produced Plan documentation.

Table 1: Question-by-Question Assessment

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
SQ13–14 Were children and 18–29-year-olds referred to VSIG?	None of 9 child deaths met VSIG criteria. 1 of 5 young adult deaths referred. Reasons cited: ‘known AE’, ‘alternative causes’, ‘insufficient information.’ ‘VSIG is not a death investigation group.’	Strategy 6 (p.5): Identification and clinical investigation of reports of special interest including deaths. Strategy 7 (p.5): Causality assessment of AEFI of special interest.	‘TGA will actively and comprehensively monitor any COVID-19 vaccines for safety’ (p.13). ‘Pharmacovigilance Plan’ framework anticipated (p.13, 15).	Plan committed to investigating deaths as ‘reports of special interest.’ TGA’s own VSIG Work Instruction (FOI 4029-06, pp.3–4) states youth deaths ‘had potential to change benefit-risk balance.’ 0/9 children referred despite meeting internal criteria. National policy requires ‘comprehensive’ monitoring; selective application does not meet this standard.	ICH E2E §2.1.2(b) (p.4): Safety Specification must address populations not studied in pre-approval phase, including children. ICH E2E §3.1.3 (p.7): Action plan for each safety issue must include monitoring and milestones for evaluation.
SQ15–16 Do causality assessment reports exist? Provide copies.	Refused. ‘TGA does not disclose details of individual cases to ensure patient privacy.’ Did not confirm whether reports exist. Did not address ‘if they do not exist, why not?’	Strategy 7 (p.5): Causality assessment of AEFI of special interest. Output 2.2 (p.8): Investigation and clinical assessment of AEFI of special interest.	‘Risk management plans and pharmacovigilance required’ (p.13, 15). Comprehensive monitoring commitment requires verifiable assessment processes.	Neither confirms nor denies reports exist. Redacted copies were requested. Second part of question (‘if not, why not?’) unanswered. Comprehensive review claimed but evidence withheld, rendering claim unfalsifiable.	ICH E2E §3.1.2 (p.6): Routine pharmacovigilance requires systems ensuring suspected adverse reactions ‘collected and collated in an accessible manner.’ CIOMS VIII Ch.3 (pp.13–28): Signal definition requires documented evaluation. Signal taxonomy requires classification and documented assessment.
SQ17 Why were only 2 of 160 fatal cases in 2022 referred to VSIG?	Deflects to SQ13–14. VSIG is ‘one of a number of tools.’ ‘It is not correct to suggest that all AEFI reports where an outcome of death is reported will automatically meet the criteria to convene a VSIG.’	Strategy 6 (p.5): Investigation of reports of special interest. Objective 2 (p.8): Timely detection, investigation and response to vaccine safety issues/signals.	‘Actively and comprehensively monitor’ (p.13). 1.25% referral rate (2/160) for deaths during provisional approval is inconsistent with ‘comprehensive’ commitment.	If VSIG is ‘one tool,’ what tool was used for the other 158 deaths? No documented alternative assessment pathway produced. Plan committed to enhanced monitoring beyond routine.	ICH E2E §3.1.3 (p.7): Each safety issue requires documented objective, action, rationale, and milestones. CIOMS VIII Ch.4 (pp.29–56): All relevant data sources to be searched systematically with documented processes.
SQ18 How many of 1,053 DAEN deaths have had causality assessments?	‘Please refer to response to questions 13 and 14.’ No number provided.	Strategy 7 (p.5): Causality assessment of AEFI of special interest. Output 2.2 (p.8): Investigation and clinical assessment capacity.	‘Actively and comprehensively monitor’ (p.13). If comprehensive monitoring occurred, a count of completed causality assessments should be readily available.	Direct question not answered. If systematic causality assessment occurred under the Plan, TGA should be able to state a figure. Consistent with Oct 2025 Senate Estimates admission that monitoring was not tracked against Plan objectives.	ICH E2E §3.1.4 (p.7): ‘Summary of actions to be completed, including milestones’ — requires documented tracking of pharmacovigilance actions. CIOMS VIII Ch.3 (p.14): Signal taxonomy requires documented classification of each identified signal with sufficient evidence to justify verifiatory action.
SQ19 How many of 1,053 deaths	‘Please refer to response to questions 13 and 14.’	Output 3.1 (p.10): Timely regulatory action and risk communication.	‘TGA responsible for comprehensive post-market monitoring’	~50 of 1,053 = 4.7% expert review rate. Weekly reports cited as access point contain public communications	ICH E2E §3.1.3 (p.7): Requires monitoring and milestones for each safety issue.

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
referred to VSIG? Where can reports be accessed?	Partial: 15 VSIGs reviewed 'over 50 cases.' Access: weekly safety report URLs listed.	Objective 3 (p.10): Appropriate regulatory and risk minimisation responses.	(p.6, 13). 4.7% expert review rate is not consistent with comprehensive commitment.	summaries, not individual case assessments or audit trails. 95.3% of deaths have no documented expert assessment pathway.	ICH E2E Annex §3 (pp.10–11): Active surveillance must 'ascertain completely the number of adverse events via a continuous pre-organised process.'

Table 2: FOI Evidence — Capacity vs Application

FOI documents demonstrate TGA had operational expert capacity and was actively briefing on Plan strategies while restricting expert causality assessment of the most serious adverse events.

FOI Document	Content	Relevance to SQ13–19	Plan / Policy Gap	Audit Finding
FOI 4029-06 VSIG Work Instruction (Jan 2019)	Selective criteria: serious, unexpected, no obvious non-vaccine cause (pp.3–4). States young deaths 'had potential to change favourable benefit-risk balance.'	Contradicts SQ13–14 response. Internal criteria identify youth deaths as meeting threshold, yet TGA tells Senate they did not meet criteria.	Strategy 6–7 (p.5): deaths as 'reports of special interest.' National Policy (p.13): 'comprehensive' monitoring. Selective application inconsistent with both.	Internal criteria contradicted by practice. Criteria written, not applied to 9 child deaths.
FOI 4029-05 VSIG Meeting (Sep 2022)	VSIG convened and operational. Reviewed fatal myocarditis in 21-year-old. Comprehensive causality assessment. Regulatory recommendations issued.	Expert panel operational Sep 2022 — same year 6 children died without VSIG referral. Capacity existed and was used for fatal case in same period.	Output 2.2 (p.8): investigation capacity demonstrated for 21yo death but not applied for child deaths in same period. Policy (p.13): comprehensive.	Policy decision, not capacity limitation. Panel assessed 21yo fatal case; 6 child deaths in same year not referred.
FOI 4029-03 Plan strategy briefing (Sep 2021)	TGA actively briefing advisory bodies on Safety Monitoring Plan strategies during rollout.	TGA presenting Plan strategies to advisory bodies while not systematically implementing them for death investigations.	Plan briefed as operational framework while deaths handled through undocumented 'day-to-day processes.' Policy commitment not operationalised.	Knowledge of Plan + capacity + non-application = deliberate policy choice.
FOI 5082 COVID-19 enhancements	COVID-19 specific enhancements to TGA infrastructure and processes documented.	Enhanced infrastructure built for COVID-19. Capability existed to systematically track death investigations.	Infrastructure not integrated with Plan framework for systematic death assessment. Plan and Policy both require demonstrable delivery.	Enhanced infrastructure + selective use = documentation gaps reflect policy decisions, not resource constraints.

Table 3: Prior SQON Pattern — Consistent Non-Disclosure

TGA's December 2025 response cites 9 prior SQONs as evidence it has already addressed vaccine death questions. None produced Plan implementation records. If systematic documentation existed, at least one of these 9 responses would have produced it.

SQ / FOI Reference	Date	TGA Response Pattern	Plan Documentation Produced?	Relevance to SQ13–19
SQ24-000234 (FOI 4263)	Apr 2024	Cause of death determinations: aggregate statistics. 'Low threshold reporting; few causally linked.'	No	SQ18 asks same question. Still unanswered.
SQ24-000233 (FOI 4205)	Apr 2024	DAEN deaths database: aggregate statistics only.	No	SQ19 asks for access. Still only public communications available.
SQ23-002226	Jan 2024	'Reporting does not equal causation.' Deflects from assessment obligation.	No	Same deflection used in SQ17 response.
SQ23-001101	Sep 2023	Single death follow-up: case-specific response without system-level evidence.	No	Individual case handling does not demonstrate systematic framework.
SQ23-000139	Aug 2023	Causality of 7- and 9-year-old deaths: confirmed but VSIG referral not addressed.	No	Same child deaths, same non-referral. Pattern repeated in Dec 2025.
SQ23-001074	Aug 2023	Temporal association / excess deaths: statistical deflection.	No	Signal assessment records not produced.
SQ23-001086 (FOI 4217)	Jul 2023	Child deaths under 18: 'No new safety concerns' without documented expert assessment.	No	Conclusion asserted without verifiable process. Directly relevant to SQ13.
SQ23-001134	Jul 2023	Child vaccine deaths: refers to prior responses.	No	Circular referral. Same pattern repeated Dec 2025.
SQ24-000230	Apr 2024	COVID-19 vaccine autopsies: limited information.	No	Autopsy integration into signal management unaddressed.

Governance Failure Analysis

The structural and systemic issues identified in TGA's pharmacovigilance response constitute governance failure under ANAO, VAGO, and ISO assessment criteria. The evidence demonstrates inadequate design and operation of controls against commitments.

1. Child deaths: internal criteria contradicted by practice

9 child deaths. Zero referred to VSIG. TGA's VSIG Work Instruction (FOI 4029-06, pp.3–4) states youth deaths 'had potential to change the favourable benefit-risk balance.' Plan Strategy 6 (p.5) committed to investigating deaths as 'reports of special interest.' National policy (p.13) required 'comprehensive' monitoring. ICH E2E §2.1.2(b) (p.4) requires children be addressed as an unstudied population. TGA wrote the criteria, published the Plan, operated under the policy, adopted the standard, then did not apply any of them to child deaths.

2. Unanswered question: how many causality assessments?

SQ18 is a direct, quantifiable question. TGA deflects without providing a number. If systematic causality assessment occurred under Output 2.2 (p.8), a count should be readily available. ICH E2E §3.1.4 (p.7) requires a summary of actions completed with milestones. If completed, this summary would answer SQ18 immediately. The inability or refusal to provide a figure is consistent with the October 2025 Senate Estimates admission that monitoring was conducted as 'day-to-day processes' not tracked against Plan objectives. Under ANAO performance audit methodology, this represents incomplete performance indicators and undocumented risk assessments.

3. Expert review rate: 4.7%

Approximately 50 of 1,053 reported deaths reviewed by VSIG. TGA describes VSIG as 'one of a number of tools.' No documented alternative assessment pathway has been produced for the remaining 95.3%. Plan Objective 2 (p.8) committed to timely detection, investigation and response. National policy (p.13) required comprehensive monitoring. ICH E2E §3.1.3 (p.7) requires each safety issue have a documented action plan. The gap is not that VSIG was not always convened. The gap is that no documented process exists for the 95.3% not reviewed by any expert panel. Under ISO 31000, a risk treatment applied to 4.7% of identified risks does not constitute an integrated risk management framework.

4. Capacity existed; application was selective

FOI 4029-05: VSIG operational September 2022, assessing a fatal case in a 21-year-old. Same year, 6 children died without referral. FOI 4029-03: TGA briefing advisory bodies on Plan strategies September 2021. FOI 5082: COVID-19 specific enhancements documented. The infrastructure existed, the capacity existed, the Plan was being briefed as operational. Child deaths were not referred. This is a control design failure: the activity layer does not deliver the framework requirements. Under VAGO performance audit standards, this represents methodology lacking rigour and risks not addressed in a timely manner.

5. Unfalsifiable claims

SQ15–16 asked for causality reports. TGA refused, citing privacy. The question also asked: 'if they do not exist, why not?' This was not answered. TGA simultaneously claims all serious AEFIs are reviewed, withholds evidence of that review, and declines to confirm whether the evidence exists. ICH E2E §3.1.2 (p.6) requires information collected 'in an accessible manner.' Under ANAO audit criteria, claims not supported by documentary evidence cannot be accepted as sufficient. The comprehensive review claim is unfalsifiable by design.

6. Serial deflection across 9 prior SQONs

TGA cites 9 prior SQONs (July 2023 – April 2024) as evidence it has already addressed death-related questions. Analysis of each shows the same pattern: aggregate statistics, privacy deflection, case-specific evasion, circular referral. Across 9 opportunities over two years, TGA did not produce Plan implementation records, signal management audit trails, or systematic evidence of enhanced monitoring delivery. SQ23-000139 (August 2023) asked specifically about 7- and 9-year-old deaths. Two years later, SQ13 asks the same question. The answer is unchanged. Repetition without

documentation is itself evidence that the documentation does not exist. Under ISO 19011, an auditable process must produce verifiable records. The absence of records across 9 Senate questions constitutes sufficient evidence of non-implementation.

Conclusion

Senator Antic asked seven direct questions. TGA's response:

1. Confirmed zero of 9 child deaths referred for expert review, contradicting TGA's own published criteria
2. Did not state how many of 1,053 deaths received formal causality assessments
3. Refused to release causality reports and did not confirm whether they exist
4. Disclosed a 4.7% expert review rate (approximately 50 of 1,053 deaths)
5. Cited weekly safety reports as evidence of systematic monitoring — reports containing public communications summaries, not implementation documentation
6. Cited 9 prior SQONs as 'previous answers' — none of which produced Plan documentation

Each finding maps to documented gaps against the COVID-19 Vaccine Safety Monitoring Plan (February 2021), the National Cabinet-endorsed Australian COVID-19 Vaccination Policy (November 2020), and the international pharmacovigilance standards Australia adopted as reference framework.

The questions Senator Antic asked in December 2025 are the same questions asked in July 2023. The responses are unchanged. Two years of Senate scrutiny have not produced a single Plan implementation record. The documentation either does not exist or has not been disclosed. Both conclusions confirm the audit's central finding: the enhanced monitoring framework promised under Cabinet-endorsed policy and required for provisional approval cannot be verified through documentation.

References

Senate and Parliamentary

- Senate Questions on Notice, December 2025–26 (Senator Antic, SQ13–19)
- Prior SQONs: SQ24-000230, SQ24-000234 (FOI 4263), SQ24-000233 (FOI 4205), SQ23-002226, SQ23-001101, SQ23-000139, SQ23-001074, SQ23-001086 (FOI 4217), SQ23-001134 https://www.aph.gov.au/Parliamentary_Business/Senate_Estimates/ca/budgetestimates
- Senate Estimates testimony, 9 October 2025 (Community Affairs Legislation Committee)

National Policy

- Australian COVID-19 Vaccination Policy, 13 November 2020 (pp.6, 13, 15) <https://www.health.gov.au/sites/default/files/documents/2020/12/covid-19-vaccination-australian-covid-19-vaccination-policy.pdf>

TGA Documents

- COVID-19 Vaccine Safety Monitoring Plan, February 2021 (Strategies 6–7 p.5; Output 2.2 p.8; Output 3.1 p.10; Objective 2 p.8; Objective 3 p.10) <https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf>
- FOI 4029-06: VSIG Work Instruction (January 2019), pp.3–4 <https://www.tga.gov.au/sites/default/files/2023-01/foi-4029-06.PDF>
- FOI 4029-05: VSIG Meeting Minutes (September 2022) <https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/reference-documents/foi-4029/foi-4029-05.PDF>
- FOI 4029-03: Plan strategy briefing (September 2021) <https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/tree/main/reference-documents/foi-4029>
- FOI 5082: COVID-19 enhancements documentation <https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/reference-documents/FOI5082.pdf>
- FOI 4217: Deaths of children under 18 <https://www.tga.gov.au/sites/default/files/2023-05/foi-4217-01.pdf>

Regulatory and Oversight

- OAIC Decision [2025] AICmr 54 (26 March 2025) <https://www.austlii.edu.au>
- NCIRS, Surveillance of adverse events following immunisation in Australia, COVID-19 vaccines, 2022

International Standards

- ICH E2E: Pharmacovigilance Planning (18 November 2004) §2.1.2(b) p.4: populations not studied (children); §3.1.2 p.6: routine pharmacovigilance systems; §3.1.3 p.7: action plan for safety issues; §3.1.4 p.7: summary of actions and milestones; Annex §3 pp.10–11: active surveillance https://database.ich.org/sites/default/files/E2E_Guideline.pdf
- CIOMS Working Group VIII: Practical Aspects of Signal Detection in Pharmacovigilance (2010) Ch.3 pp.13–28: signal definition and taxonomy; Ch.4 pp.29–56: data sources for signal detection; Ch.5 pp.57–78: signal detection methodologies <https://cioms.ch/publications/product/practical-aspects-of-signal-detection-in-pharmacovigilance-report-of-cioms-working-group-viii/>

Audit Standards

- ANAO: Performance Audit Process and Commonwealth Performance Series (CPS 107) <https://www.anao.gov.au/work/insights/performance-audit-process>
- Public Governance, Performance and Accountability Act 2013 (PGPA Act), s.17 <https://www.legislation.gov.au/Details/C2013A00123>
- ISO 19011:2018 – Guidelines for Auditing Management Systems <https://www.iso.org/standard/70017.html>
- ISO 31000:2018 – Risk Management <https://www.iso.org/standard/65694.html>
- VAGO: Performance Audit Standards <https://www.audit.vic.gov.au>

Audit Repository

- Rekaris, P. (2026). Documentation Gap Analysis: Implementation Audit of TGA COVID-19 Vaccine Safety Monitoring Plan <https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit> <https://doi.org/10.5281/zenodo.17731054>